Green Zone MO Guide

Right patient. Right zone. Right time.



01 Patients move one way

MTS level 4 and 5 patients, from triage to discharge

1. **Triage**
   MTS level 4 or 5
2. **Secondary triage**
3. **ECG, irrigation, dressing**
   Only if needed
4. **You, the MO**
   Fill in the green slip, attach the queue number
5. **Red Box**
   The slip goes in; the patient waits in the tent
6. **Treatment room (PPP)**
   Calls the number, does each order, documents it

Results pending, observation or referral

PPP takes the patient to the CCA (Chair Centric Area) and hands over to Yellow Zone Extension staff, who record vital signs on the slip and send the patient back to you.

To finish

Results back: call the queue number again. Attach the slip to the BHT, complete the notes, give a TCA from the appointment flow chart and discharge advice.

02 Three rules to keep

NEVER WAITS OUTSIDE

Bring them inside

Elderly · **NSTEMI or ACS** · **Stat drugs and monitoring**

- Seat them in the CCA, the rooms in front of R2, the R2 corridor or any zone with space
- The designated MA and HO there start management and monitoring
- No space: tell the EP in charge

ADMIT, DISCHARGE, TRANSFER

Reassess first

- Reassess every patient; review every blood result and image
- Came within 24 hours: ask the TL, a master student or the EP to look at the case
- CT brain and going home: EP review, then an EMWATCH or EMACC appointment

WHEN IT STALLS

Call the EP in charge

- Several disciplines and no disposition after a long wait: inform the EP
- External referral: discuss with the EP before accepting
- Pending at the end of your shift: hand over to the next MO and document who

03 Numbers to know

EMACC follow-up
`ext 7040`
Via 03-2615 5555, MO in charge

MAU mobile team
`ext 6682 / 6684`
After hours: MAU 1, 2 and 3 wards

IJN hotline
`03-2617 8996`
ACS meeting the referral criteria

Neuro Rehab Clinic
`ext 7099`
Mild TBI, normal CT, PTA over 4 h

MAU FAST clinic
`MHKL level 2`
8 am–4:30 pm; CAP, CURB-65 0–1

Open the app
Every pathway and rule, step by step

Sources: Clinical Pathway in Emergency for MO HKL (ebook by the HKL Emergency Physicians); Green Zone memos 2022 to 2026; EMACC SOP 2018. A reference, not a substitute for clinical judgement: follow current department guidance. No patient data.

Pathways, follow-up and referrals

04 The 23 ebook pathways

Admit · Observation · Discharge · Discharge advice

Listed by page in Clinical Pathway in Emergency for MO HKL. The colours are the ebook's outcome boxes.

5-9	Acute behavioural disturbance	27	Community acquired pneumonia	43-44	Hypoglycaemia
10-11	Abdominal pain	28-29	CVA and TIA	45-50	Pain management
12-14	Acute coronary syndrome and Obsess protocol	30	Acute cellulitis	51-52	Paracetamol overdose
15-16	Anaphylaxis	31-32	Dengue fever	53-54	Renal colic
17-18	Acute gastroenteritis (adult)	33-35	Deep vein thrombosis	55-56	Supraventricular tachycardia
19-23	Bronchial asthma	36-37	Heart failure	57-58	Syncope
24	Biliary disease	38-40	Hypertensive urgency	59-63	Acute scrotal pain
25-26	Mild traumatic brain injury	41-42	Hyperglycaemia		

05 EMACC follow-up

ext 7040 · Green Zone, Mon–Fri 9 am–4 pm · first review within 1 week

For adults 18+ being discharged, with no own-clinic appointment within 14 days, a case below, and no admission needed.

Case	Review in EMACC
Hypertensive urgency	Within 1 week for BP review; renal profile if started on captopril
Low-risk chest pain after OBSSES	Within 2 weeks: symptoms, repeat ECG, EST result; aspirin 150 mg OD if no contraindication; EST within 1 week, or refer to PC if EST is contraindicated
Uncontrolled DM: sugar over 11, not DKA or HHS, no admission needed	Within 1 week if there is no timely primary clinic appointment
Heart failure improved with ED treatment, no admission needed	Within 1 week; repeat renal profile if frusemide or an ACE inhibitor was started; suspected new HF: also refer to PC
Renal colic, improved, normal renal profile	First episode with a normal KUB X-ray: EMACC review. Recurrent: KUB ultrasound, then EMACC reviews symptoms and result
Non-specific abdominal pain: 7 days or less, no diagnosis, normal workup, improved	Within 3 days; discuss every case with the EP in charge
Trauma: up to 2 uncomplicated rib fractures, undisplaced closed fracture, minor dislocation, suspected fracture with a normal X-ray, soft tissue injury with marked swelling, large laceration (especially in diabetics)	Within 3 days for a wound check and pain review; repeat X-ray if needed; MC until the EMACC appointment
Blood results to review, such as TFT	As arranged

06 Asthma Zone

Secondary triage: A look, listen, feel; B focused history (asthma or COPD?); C rapid severity check.

Critical? — **Yes:** uptriage to the Red or Amber Zone

No: the AMO gives up to 2 nebulisers. Record BP, HR, RR, SpO2 and PEFR after each. Urgent FBC or ABG needed: uptriage.

The **Amber Zone MO reassesses** every patient.

Recovered?

- **No:** uptriage to Amber or Red, or admit
- **Yes:** record the risk factors; discharge with inhaler and medicine review, follow-up at OPD, PC or KK, and the asthma card updated

07 Refer to

- **Joint dislocation**, shoulder too: the on-call Orthopaedic team, not the clinic.
- **GI bleed:** medical cause, Gastroenterology; surgical cause, General Surgery. **Cholecystitis** without jaundice: General Surgery. **Jaundice:** Gastroenterology.
- **Medical ward:** refer to the Medical specialist or MO on call first.
- **Psychiatric relapse**, known history, no other medical problem: Psychiatry MO on call, no bloods or CT first.
- **Gout:** nearest Klinik Kesihatan; complex cases to Rheumatology.
- **Dengue from a KK**, warning signs only, normal CCTVR and vitals, platelets over 50,000, not morbidly obese: straight to the Observation Ward.
- **IPR (respiratory):** stable, no inotropes, Venturi 40% at most.

Sources: the ebook above; EMACC SOP (1 Feb 2018: check the extension and hours are current); Asthma Zone memo; referral memos and letters 2018 to 2026. A reference, not a substitute for clinical judgement. No patient data.